

Analyzing Reproductive Health and Behavior Patterns Using NSFG 2022–2023

I. Introduction

This analysis utilizes data from the 2022-2023 National Survey of Family Growth (NSFG), focusing specifically on female respondents. The NSFG is a comprehensive survey that collects information on demographics, family structures, pregnancy and birth history, marital and relationship history, fertility, contraceptive history, and health status.

This analysis aims to use exploratory data analysis techniques to examine the trends related to demographics, lifestyle choices, and overall health and how they impact sexual and reproductive health behavior of women.

II. The Dataset and Variables

1. Variable Selection

The dataset was retrieved from the 2022-2023 NSFG Public use data files on Female respondents. The raw dataset had 5,586 observations and 1,912 variables. Variables applicable to most if not all respondents were selected to provide insights on the respondents' demographics, lifestyle, and health status.

The following variables were selected for this analysis:

- AGER - Respondent's age at interview
- RSCRRACE - Respondent's race as reported in screener
- HHKIDS18 - No. of children < 19 years in household
- FMARITAL - Formal (legal) marital status relative to opposite-sex spouses
- HIEDUC - Highest completed year of school or highest degree received
- LIFPRTNR - No. of opposite-sex sexual partners in lifetime
- GENHEALT - Respondent's general health
- CONSTAT1 - Current contraceptive status (1st priority)
- PREGNUM - Total no. of pregnancies
- ASKSMOKE - Asked Respondent whether they smoke cigarettes or other tobacco
- DRINK12 - How often drank alcoholic beverages in the last 12 mos.

2. Data Pre-processing

Missing values were handled by median imputation. Values entered as “Refused” and “Don’t Know” are recoded and handled as missing values. Categorical variables (RSCRRACE, HIEDUC, FMARITAL, GENHEALT, CONSTAT1, ASKSMOKE, DRINK12, DIABETES, PCOS) are recoded as factors. To lessen the number of levels in CONSTAT1 variable, similar items were grouped under broader categories (see detailed list in Appendix).

III. Results

Statistical Analyses

1. Poisson Regression

A Poisson regression model is used to predict the number of opposite-sex sexual partners in lifetime (LIFPRTNR) using demographic and behavioral data (age, education, smoking, alcohol, pregnancy, and no. of kids.).

$$\log(\lambda) = \beta_0 + \beta_1 \cdot \log(\text{AGER}) + \beta_2 \cdot \text{ASKSMOKE} + \beta_3 \cdot \text{DRINK12} + \beta_4 \cdot \text{HHKIDS18} + \beta_5 \cdot \text{HIEDUC} + \beta_6 \cdot \text{PREGNUM}$$

Where λ is the expected count, β_0 is the intercept, and $\beta_1, \beta_2, \dots, \beta_6$ are the coefficients for each predictor variable.

Table 1

Poisson regression model summary

```
Call:
glm(formula = LIFPRTNR ~ log(AGER) + ASKSMOKE + DRINK12 + HHKIDS18 +
    HIEDUC + PREGNUM, family = poisson, data = data_train)

Coefficients:
              Estimate Std. Error z value Pr(>|z|)
(Intercept) -2.737500    0.085449  -32.037 < 2e-16 ***
log(AGER)    1.111386    0.024545   45.280 < 2e-16 ***
ASKSMOKE5   -0.186118    0.015659  -11.886 < 2e-16 ***
DRINK122     0.138291    0.019873   6.959 3.43e-12 ***
DRINK123     0.391225    0.019279  20.292 < 2e-16 ***
DRINK124     0.311492    0.020708  15.042 < 2e-16 ***
DRINK125     0.471504    0.018574  25.386 < 2e-16 ***
DRINK126     0.763237    0.022925  33.293 < 2e-16 ***
HHKIDS18    -0.081200    0.006050  -13.421 < 2e-16 ***
HIEDUC2      0.502526    0.045208  11.116 < 2e-16 ***
HIEDUC3      0.796821    0.039902  19.970 < 2e-16 ***
HIEDUC4      0.378944    0.032801  11.553 < 2e-16 ***
HIEDUC5      0.639266    0.030982  20.634 < 2e-16 ***
HIEDUC6      0.564236    0.035188  16.035 < 2e-16 ***
HIEDUC7      0.597884    0.036620  16.326 < 2e-16 ***
HIEDUC8      0.468323    0.031098  15.060 < 2e-16 ***
HIEDUC9      0.318016    0.033093   9.610 < 2e-16 ***
HIEDUC10     0.335102    0.044957   7.454 9.07e-14 ***
HIEDUC11     0.201424    0.056755   3.549 0.000387 ***
PREGNUM      0.079167    0.003461  22.875 < 2e-16 ***
---
Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1

(Dispersion parameter for poisson family taken to be 1)

    Null deviance: 50060  on 4467  degrees of freedom
Residual deviance: 41477  on 4448  degrees of freedom
AIC: 53817

Number of Fisher Scoring iterations: 6
```

To determine the validity of the results, the assumptions are checked as follows:

- i. Count Outcome
The dependent variable, LIFPRTNR, is a count variable (0, 1, 2,...), hence, this assumption is met.
- ii. Independence of Observations
Each row in the dataset represents a unique respondent, hence the observations are independent.
- iii. Equidispersion

```
Mean: 6.800895
Var: 112.1817
Overdispersion test

data: poisson_model
z = 17.669, p-value < 2.2e-16
alternative hypothesis: true dispersion is greater than 1
sample estimates:
dispersion
14.74234
```

The mean and variance are not equal with variance being much greater than the mean. Overdispersion is observed.

iv. Linear Relationship between Predictors and Log Count

Figure 1

Residuals vs. Fitted plot

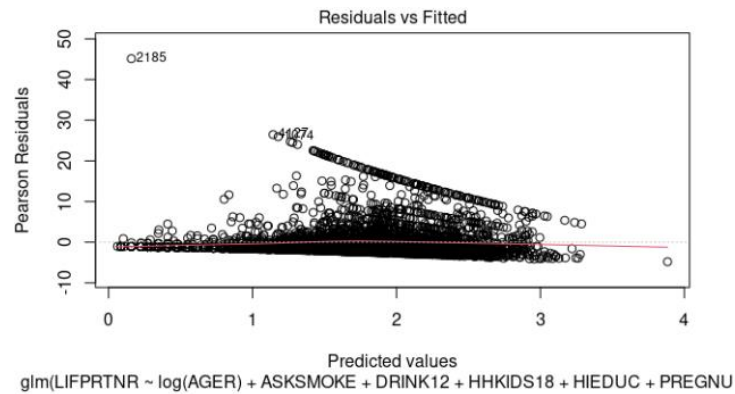


Figure 1 shows that most residuals are scattered randomly around 0. However, some exhibit a downward trend, which indicates some violation of this assumption.

v. No Multicollinearity

Table 2

VIF table for predictors

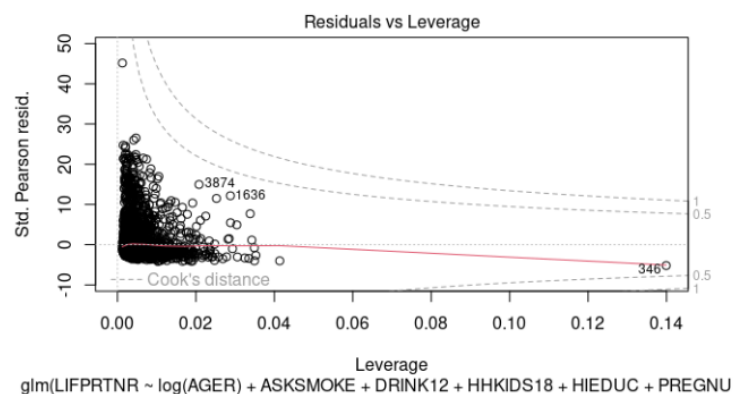
	GVIF	Df	GVIF ^{1/(2*Df)}
log(AGER)	1.298801	1	1.139650
ASKSMOKE	1.050631	1	1.025003
DRINK12	1.169132	5	1.015749
HHKIDS18	1.364404	1	1.168077
HIEDUC	1.334654	10	1.014538
PREGNUM	1.619450	1	1.272576

The Variance Inflation Factor (VIF) for each predictor is near 1 which indicates moderate correlation. Hence, this assumption is met.

vi. No Extreme Outliers

Figure 2

Residuals vs. Leverage plot



Although a few deviations from other fitted values are observed, none are extreme enough to go past the Cook's distance lines. Hence, this assumption is met.

The Poisson model failed the assumptions for linearity and equidispersion and showed a high level of overdispersion which implies poor fit. An alternative approach would be to use negative binomial regression for overdispersed data.

Table 3*Negative Binomial Regression Model Summary*

```

Call:
glm.nb(formula = LIFPRN ~ log(AGER) + ASKSMOKE + DRINK12 +
      HHKIDS18 + HIEDUC + PREGNUM, data = data_train, init.theta = 0.7410732654,
      link = log)

Coefficients:
              Estimate Std. Error z value Pr(>|z|)
(Intercept) -4.37281      0.24681 -17.717 < 2e-16 ***
log(AGER)    1.51192      0.07680  19.685 < 2e-16 ***
ASKSMOKE5   -0.16125      0.04691  -3.438 0.000587 ***
DRINK122     0.23363      0.05896   3.963 7.41e-05 ***
DRINK123     0.48000      0.06012   7.984 1.42e-15 ***
DRINK124     0.42548      0.06443   6.604 4.02e-11 ***
DRINK125     0.59044      0.05974   9.883 < 2e-16 ***
DRINK126     0.82588      0.08786   9.399 < 2e-16 ***
HHKIDS18    -0.06784      0.02173  -3.121 0.001800 **
HIEDUC2      0.66781      0.12557   5.318 1.05e-07 ***
HIEDUC3      0.91494      0.13062   7.004 2.48e-12 ***
HIEDUC4      0.46707      0.08328   5.608 2.04e-08 ***
HIEDUC5      0.71569      0.08241   8.685 < 2e-16 ***
HIEDUC6      0.69490      0.10245   6.783 1.18e-11 ***
HIEDUC7      0.68788      0.10871   6.328 2.49e-10 ***
HIEDUC8      0.59207      0.08275   7.155 8.35e-13 ***
HIEDUC9      0.43931      0.09066   4.846 1.26e-06 ***
HIEDUC10     0.43720      0.13778   3.173 0.001507 **
HIEDUC11     0.16962      0.17495   0.970 0.332273
PREGNUM      0.10358      0.01433   7.229 4.86e-13 ***
---
Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1

(Dispersion parameter for Negative Binomial(0.7411) family taken to be 1)

Null deviance: 6050.6  on 4467  degrees of freedom
Residual deviance: 4895.2  on 4448  degrees of freedom
AIC: 25189

Number of Fisher Scoring iterations: 1

              Theta:  0.7411
            Std. Err.:  0.0180

2 x log-likelihood:  -25147.0270

```

This model reported a lower AIC value compared to the Poisson model, which indicates better fit.

2. Contingency Tables

A contingency table is used to analyze the counts of contraceptive methods (CONSTAT1) and marital status (FMARITAL) and determine if there is an association between these factors.

Table 4*Contingency Table of Contraceptive Method and Marital Status and Chi-squared Test*

```

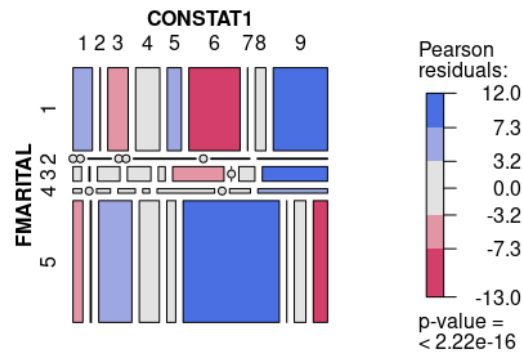
              CONSTAT1   1   2   3   4   5   6   7   8   9  10
FMARITAL
1              205    5  216  255  150  279  262    3  112  573
2               0     0    3    0    0    1    7    0    5    9
3              16    2   42   44   13   22   74    0   30  122
4               5     0    8   10    4    8   25    0   12   40
5             153   16  513  311  138  147 1343    1  180  221
Warning in chisq.test(contable, correct = FALSE) :
  Chi-squared approximation may be incorrect

Pearson's Chi-squared test

data:  contable
X-squared = 1066.9, df = 36, p-value < 2.2e-16

```

There is a statistically significant association between contraceptive method (CONSTAT1) and marital status (FMARITAL) ($p < 0.05$). The high X-squared value implies a strong association.

Figure 3*Mosaic plot of Contraceptive Methods and Marital Status*

3. Ordinal Logistic Regression

Ordinal logistic regression (OLR) is used to predict womens' self-reported general health status (GENHEALT) using age, smoking status, drinking habits, and number of pregnancies (AGER , ASKSMOKE , DRINK12, PREGNUM).

Table 5*Ordinal Logistic Regression Model Summary*

```
call:
polr(formula = GENHEALT ~ AGER + DRINK12 + ASKSMOKE + PREGNUM,
      data = data_train, Hess = TRUE)

Coefficients:
              Value Std. Error t value
AGER          0.01312   0.003268  4.0160
DRINK122      0.36876   0.084399  4.3692
DRINK123      0.37452   0.085522  4.3792
DRINK124      0.25381   0.091602  2.7708
DRINK125     -0.17030   0.084186 -2.0229
DRINK126     -0.07018   0.129687 -0.5412
ASKSMOKE5    -0.22169   0.067733 -3.2730
PREGNUM       0.04177   0.016975  2.4606

Intercepts:
      Value Std. Error t value
1|2 -0.7522   0.1095  -6.8666
2|3  0.9135   0.1096   8.3358
3|4  2.7032   0.1171  23.0767
4|5  4.5338   0.1529  29.6469

Residual Deviance: 11972.60
AIC: 11996.60
```

Table 6*Odds Ratios of Predictors of General Health*

	β	Odds Ratio (e^{β})
AGER	0.01312	1.0132
DRINK122	0.36876	1.4459
DRINK123	0.37452	1.4543
DRINK124	0.25381	1.2889
DRINK125	-0.17030	0.8434
DRINK126	-0.07018	0.9322
ASKSMOKE5	-0.22169	0.8012
PREGNUM	0.04177	1.0427

The odds ratios given by e^{β} (where β is the coefficient) are given in Table 6. The positive coefficients result in higher odds of being in lower categories (poor health) while negative coefficients result in higher odds of being in higher categories (good health).

Model Evaluation using McFadden's Pseudo R2

```
fitting null model for pseudo-r2
McFadden
0.009452265
```

The model has a McFadden's R2 value of 0.00945 which indicates a weak model fit. The predictors likely do not predict self-rated general health well.

4. Multinomial Logistic Regression

Multinomial logistic regression (MLR) is used to predict marital status (FMARITAL) using number of pregnancies, age, and number of opposite-sex sexual partners (PREGNUM, AGER, LIFPRTNR).

Table 7

Multinomial Logistic Regression Model Summary

```
Call:
vglm(formula = FMARITAL ~ PREGNUM + AGER + LIFPRTNR, family = multinomial,
data = data_train)

Coefficients:
              Estimate Std. Error z value Pr(>|z|)
(Intercept):1  -4.414273   0.160426  -27.516 < 2e-16 ***
(Intercept):2 -14.550266   1.945742  -7.478 7.55e-14 ***
(Intercept):3  -8.192545   0.374862 -21.855 < 2e-16 ***
(Intercept):4  -7.501019   0.551370 -13.604 < 2e-16 ***
PREGNUM:1       0.328507   0.024446  13.438 < 2e-16 ***
PREGNUM:2       0.447072   0.108348   4.126 3.69e-05 ***
PREGNUM:3       0.355406   0.036271   9.799 < 2e-16 ***
PREGNUM:4       0.471356   0.050845   9.270 < 2e-16 ***
AGER:1          0.112917   0.004851  23.277 < 2e-16 ***
AGER:2          0.240504   0.044580   5.395 6.86e-08 ***
AGER:3          0.157476   0.009617  16.375 < 2e-16 ***
AGER:4          0.102746   0.014929   6.882 5.89e-12 ***
LIFPRTNR:1     -0.019386   0.003708  -5.228 1.72e-07 ***
LIFPRTNR:2     -0.003954   0.018896  -0.209 0.8343
LIFPRTNR:3      0.011311   0.004875   2.320 0.0203 *
LIFPRTNR:4      0.004551   0.008342   0.546 0.5853
---
Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1

Names of linear predictors: log(mu[,1]/mu[,5]), log(mu[,2]/mu[,5]),
log(mu[,3]/mu[,5]), log(mu[,4]/mu[,5])

Residual deviance: 7116.622 on 17856 degrees of freedom

Log-likelihood: -3558.311 on 17856 degrees of freedom

Number of Fisher scoring iterations: 8
```

Table 8

Odds Ratios of Predictors of Marital Status

	Coefficient	Odds Ratio (e^{β})
PREGNUM:1	0.328507	1.388893
PREGNUM:2	0.447072	1.563727
PREGNUM:3	0.355406	1.42676
PREGNUM:4	0.471356	1.602165
AGER:1	0.112917	1.119539
AGER:2	0.240504	1.27189

AGER:3	0.157476	1.170553
AGER:4	0.102746	1.10821
LIFPRTNR:1	-0.01939	0.980801
LIFPRTNR:2	-0.00395	0.996054
LIFPRTNR:3	0.011311	1.011375
LIFPRTNR:4	0.004551	1.004561

The odds ratios given by e^{β} (where β is the coefficient) are given in Table 8. The positive or negative coefficients result in higher or lower odds of being in the outcome category compared to the reference category. (FMARITAL=5 in this case)

Model Evaluation

Table 9
Confusion Matrix for MLR Predictions

Confusion Matrix and Statistics					
Prediction	Reference				
	1	2	3	4	5
1	260	0	0	0	152
2	7	0	0	0	0
3	44	0	0	0	22
4	18	0	0	0	4
5	130	0	1	0	479
Overall Statistics					
Accuracy : 0.6616					
95% CI : (0.633, 0.6893)					
No Information Rate : 0.5882					
P-Value [Acc > NIR] : 2.762e-07					
Kappa : 0.3581					
McNemar's Test P-Value : NA					
Statistics by Class:					
	Class: 1	Class: 2	Class: 3	Class: 4	Class: 5
Sensitivity	0.5664	NA	0.0000000	NA	0.7291
Specificity	0.7690	0.993733	0.9408602	0.9803	0.7152
Pos Pred Value	0.6311	NA	0.0000000	NA	0.7852
Neg Pred Value	0.7177	NA	0.9990485	NA	0.6489
Prevalence	0.4109	0.000000	0.0008953	0.0000	0.5882
Detection Rate	0.2328	0.000000	0.0000000	0.0000	0.4288
Detection Prevalence	0.3688	0.006267	0.0590868	0.0197	0.5461
Balanced Accuracy	0.6677	NA	0.4704301	NA	0.7221

The model achieved a 66.16% Accuracy which is higher than its No Information Rate of 58.82% with $p = 2.762e-07$. The 95% confidence interval for accuracy ranged from 63.3% to 68.93%, indicating statistically consistent performance. Performance across classes varied and showed a strong performance in Class 5 (Never Married) which is the dominant class while failing to predict well in minority classes.

5. EDA and Multiple Comparisons

Figure 4
Histogram of Distribution of Pregnancy Counts

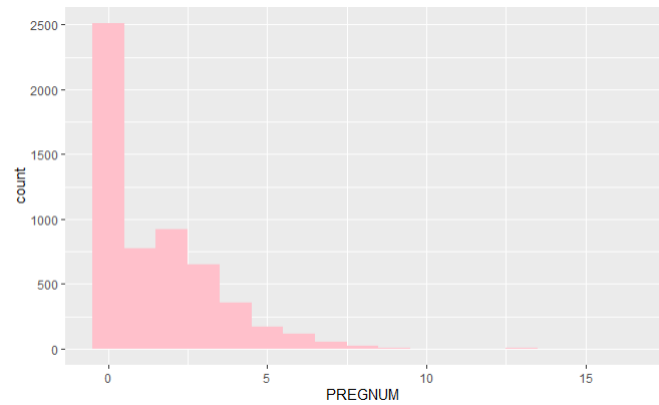


Figure 5
Bar Charts of Distribution of Races

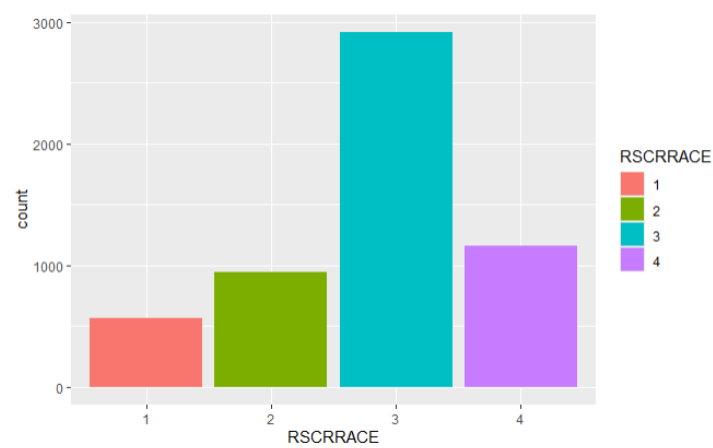
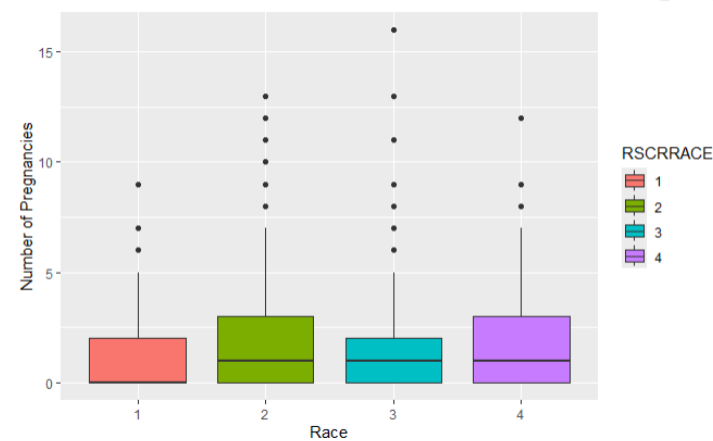


Figure 6
Boxplots of Distribution of Pregnancies across Race



Multiple comparisons is conducted to determine variations in pregnancy counts (PREGNUM) across different race groups (RSCRRACE).

Table 10
ANOVA Results Table

```
Analysis of Variance Table

Response: PREGNUM
      Df Sum Sq Mean Sq F value    Pr(>F)    
RSCRRACE      3   193.5    64.491   19.954 7.339e-13 ***
Residuals 5581 18037.7     3.232
---
Signif. codes:  0 '***' 0.001 '**' 0.01 '*' 0.05 '.' 0.1 ' ' 1
```

The ANOVA result imply a significant difference ($p < 0.05$) in pregnancy counts for different races.

Table 11
Pairwise t-tests with Bonferroni Adjustment Method

```
Pairwise comparisons using t tests with pooled SD

data: data_clean$PREGNUM and data_clean$RSCRRACE

      1      2      3
2 8.5e-12 -      -
3 0.0034 3.1e-08 -
4 9.9e-06 0.0166 0.0699

P value adjustment method: bonferroni
```

Table 12
Pairwise t-tests with FDR Adjustment Method

```
Pairwise comparisons using t tests with pooled SD

data: data_clean$PREGNUM and data_clean$RSCRRACE

      1      2      3
2 8.5e-12 -      -
3 0.00085 1.5e-08 -
4 3.3e-06 0.00332 0.01164

P value adjustment method: fdr
```

IV. Interpretations and Conclusions

Number of opposite-sex sexual partners sexual partner in lifetime

Age, more frequent alcohol consumption, higher educational attainment, and pregnancy are associated positively with the number of opposite-sex sexual partners reported in a lifetime. This suggests that as individuals get older, drink alcohol more frequently, attain higher levels of education, or have experienced pregnancy, they tend to report having had more opposite-sex sexual partners. These factors may reflect increased social exposure, opportunity, or changes in lifestyle over time

The number of kids 18 and below in household and not smoking are associated with lower partner counts. Women who have more children in the household could tend to commit to long-term relationships, limiting their number of reported lifetime partners. Since women with lower partner counts tend to avoid smoking, possibly due to having children in the household.

Contraceptive methods and marital status

There is a significant association between what type of contraception method is used and marital status. Figure 3 shows positive associations were found between sterilization and married women, sterilization and divorcees, natural methods and those who have never married. Negative associations were found between use of natural methods and married women, and sterilization and those who have never married.

The results suggest that those who are or have been in long-term opposite-sex relationships (married, divorced, or annulled) are more likely to use sterilization methods likely as a permanent method after family planning. They are also less likely to use natural methods possibly due to more education on reproductive health. In contrast, women who have never married are more likely to use natural methods and less likely to choose permanent contraception like sterilization, possibly due to future reproductive intentions.

Impact of age, pregnancies, smoking, and drinking on general health

Per additional year in age, the odds of being in poor health increases by 1.32%. For every additional pregnancy, the odds of poor health increase by 4. Compared to abstaining on alcohol, drinking once or twice a year, several times a year, and once a month increases odds of poor health by 44.59%, 45.43%, and 23.89% respectively. Strangely, drinking once a week and once a day decreases the likelihood of poor health by 15.66% and 6.78%. Compared to smoking, not smoking decreases the odds of poor health by 19.88%.

These findings suggest that aging, multiple pregnancies, and smoking have significant negative impacts on women's self-reported health status. The relationship between alcohol consumption and health appears to be complex, with moderate regular drinking showing potentially positive impact on health compared to occasional drinking. This could be due to various factors such as socioeconomic status or other lifestyle habits that weren't captured in the analysis.

Marital status, age, sexual partners, and pregnancies

Per increase in pregnancy count, the odds are 38.89% higher that the individual is married, 56.37% higher that they are widowed, 42.68% higher that they are divorced or annulled, and 60.21% higher that they are separated compared to having never married. These results suggest that women who have had more pregnancies are more likely to have been in a committed relationships at some point.

Per increase in age, the odds are 11.95% higher that the individual is married, 27.19% higher that they are widowed, 17.05% higher that they are divorced or annulled, and 10.82% higher that they are separated compared to having never married. These findings align with societal expectations where older individuals are more likely to be or have been in formal relationships.

Per increase in opposite-sex sexual partners, the odds are 1.92% lower that the individual is married, 0.4% lower that they are widowed, 1.14% higher that they are divorced or annulled, and 0.4% higher that they are separated compared to having never married. These indicate that women with more sexual partners are less likely to be currently married or widowed but more likely to be divorced or separated.

Pregnancy counts across races

Significant differences were observed in the number of pregnancies across races. Specifically, Black respondents reported significantly different pregnancy counts compared to all other groups ($p < 0.05$). Similarly, the "Others" category differed significantly from White and Hispanic groups. However, the difference between White and Hispanic participants was not statistically significant after Bonferroni correction ($p = 0.0699$). However, using the FDR adjustment method, the difference in number of pregnancies between White and Hispanic women became statistically significant ($p < 0.05$). These findings suggest that race is associated with variation in pregnancy counts, particularly among Black women.

Performing multiple tests increases the probability of incorrectly rejecting true null hypotheses (Type I error) due to multiple comparisons. To mitigate this, p-values may be adjusted using Bonferroni or False Discovery Rate (FDR) methods. The Bonferroni method

adjusts the p-values by multiplying them by the number of comparisons. This is a conservative method which reduces Type I errors but increases Type II errors (false negatives). FDR adjusts p-values to control the expected proportion of false positives among the significant results, offering a more balanced approach that maintains higher statistical power.

V. Limitations and Recommendations

Most statistical analyses performed showed significant results. Outcomes of each test confirm realistic trends and patterns regarding women's health.

The Poisson regression model for sexual partners count exhibited overdispersion which indicates poor model fit. This can be addressed through adjustment of predictors or using alternative models such as Quasipoisson and Negative Binomial. The Negative Binomial model returned a significantly lower AIC value of 25189 compared to the Poisson model (AIC: 53817) which indicates significant improvement. This may be used instead for better predictive results.

The ordinal logistic regression model for women's self-reported general health status also reported poor fit with a McFadden's R² value of 0.00945. This indicates that the predictors do not improve upon the null model significantly. Other predictors may be considered to improve model performance.

To improve overall results, additional information such as socioeconomic factors, emotional well-being, other lifestyle or habits, and family health history may be investigated.

VI. Appendix

i. Categorical Variable Values and Distribution from NSFG Code Book

RSCRRACE: Distribution	Value	n
Other	1	564
Black	2	947
White	3	2917
Hispanic	4	1158
		5586
HIEDUC: Distribution	Value	n
Less than high school completion	1	623
12th grade, no diploma	2	169
GED or equivalent	3	153
High school graduate	4	807
Some college, no degree	5	956
Associate degree: occup, tech, or voc	6	353
Associate degree: academic	7	282
Bachelor's degree	8	1267
Master's degree	9	752
Professional degree	10	135
Doctoral degree	11	89
		5586
FMARITAL: Distribution	Value	n
Married to a person of the opposite sex	1	2060
Widowed	2	25
Divorced or annulled	3	365
Separated	4	112
Never married	5	3024
		5586

GENHEALT: Distribution	Value	n
Excellent	1	1174
Very Good	2	2026
Good	3	1727
Fair	4	511
Poor	5	109
Refused	8	28
Don't Know	9	4
Inapplicable	.	7
		5586
ASKSMOKE: Distribution	Value	n
Yes	1	4281
No	5	1266
Refused	8	26
Don't know	9	12
Inapplicable	.	1
		5586
DRINK12: Distribution	Value	n
Never	1	1569
Once or twice during the year	2	975
Several times during the year	3	868
About once a month	4	753
About once a week	5	1032
About once a day	6	308
Refused	8	41
Don't Know	9	18
Inapplicable	.	22
		5586
CONSTAT1: Distribution	Value	n
Female sterilization	1	581
Male sterilization	2	314
Hormonal implant	3	141
Depo-Provera (injectable)	5	82
Pill	6	633
Contraceptive Patch	7	16
Contraceptive Ring	8	52
Emergency contraception	9	23
IUD	10	479
Diaphragm (with or w/out jelly or cream)	11	1
(Male) Condom	12	377
Female condom/vaginal pouch	13	1
Periodic abstinence: NFP, cervical mucus test or temperature rhythm	19	22
Periodic abstinence: calendar rhythm	20	96
Withdrawal	21	187
Other method	22	4
Pregnant	30	172
Seeking Pregnancy	31	252
Postpartum	32	33
Sterile--nonsurgical--female	33	313
Sterile--nonsurgical--male	34	25
Sterile--surgical--female (noncontraceptive)	35	70
Sterile--unknown reasons--male	38	1
Other nonuser--never had intercourse	40	803
Other nonuser--has had intercourse, but not in the 3 months prior to interview	41	433
Other nonuser--had intercourse in the 3 months prior to interview	42	475
		5586

New levels for CONSTAT1:

1. Surgical Sterilization: Female sterilization (1), Male sterilization (2), Sterile--surgical--female (noncontraceptive) (35)
2. Long-acting Reversible Contraceptives (LARC): Hormonal implant (3), IUD (10)
3. Hormonal Methods: Depo-Provera (injectable) (5), Pill (6), Contraceptive Patch (7), Contraceptive Ring (8)
4. Barrier Methods: (Male) Condom (12), Female condom/vaginal pouch (13), Diaphragm (11)
5. Emergency Contraception: Emergency contraception (9)
6. Natural Methods: Periodic abstinence: NFP, cervical mucus test or temperature rhythm (19), Periodic abstinence: calendar rhythm (20), Withdrawal (21)
7. Other Methods: Other method (22)
8. Non-users: Pregnant (30), Seeking Pregnancy (31), Postpartum (32), Other nonuser--never had intercourse (40), Other nonuser--has had intercourse, but not in the 3 months prior to interview (41), Other nonuser--had intercourse in the 3 months prior to interview (42)
9. Sterile (Non-surgical or Unknown): Sterile--nonsurgical--female (33), Sterile--nonsurgical--male (34), Sterile--unknown reasons--male (38)